increased food at bedtime to avoid nocturnal hypoglycaemia.

- Instruct patient to avoid exercise whenever blood glucose levels exceeds 250 mgs per day.

❖ Providing information about oral hypoglycaemic agents:

- Identify any barriers to learning, such as visual, hearing, low literacy, distractive environment.
- Teach the action, use and side effects of oral hypoglycemic agents.

❖ Maintain skin integrity:

- Maintain skin integrity.
- Use-heal protection, special mattress, foot cradles, for patients on bed rest.
- Avoid drying agents to skin.(e.g. Alcohol)
- Apply skin moisturizes to maintain supplement and prevent cracking, fissures.

❖ Improving coping strategies:

- Encourage patient and family participation is diabetes self care regimen to foster confidence.

Complication

❖ Hypoglycemia

❖ Diabetic ketoacidocis

❖ Hyperglycemic syndrome.

❖ Micro vascular complication e.g. Retinopathy, Nephropathy, Neuropathy.

❖ Micro vascular complications in Cardiovascular disease occurring both in NIDDM and IDDM.

3.8.2. Poly Cystic Ovarian Syndrome/Disease (PCOD)

Polycystic ovary syndrome (PCOS) is a disorder that includes ovulatory dysfunction, polycystic ovaries, and hyper and rogenism. It most commonly occurs in women under 30 years old and is a cause of infertility.

Causes

❖ The causes are unknown.

Signs and Symptoms

❖ Hormonal imbalance

❖ Ovulation fails and multiple fluids filled cysts

❖ Irregular menstrual periods

❖ Amenorrhea

❖ Hirsutism and obesity. (80% of women)

❖ Oligomenorrhea and infertility

❖ Insulin resistance

Management

❖ Early diagnosis (pelvic ultrasound)

❖ To improve quality of life and decrease the risk of complications

❖ OCPs are useful in regulating menstrual cycles

❖ Hirsutism may be treated with spironolactone

❖ Hyper and rogenism can be treated with flutamide and a GnRH agonist such as leuprolide

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